

[SYNTHETIC SAMPLE] Fictional record generated for the DELPHi prototype demo. Names, institutions, and statements are invented; the layout imitates a generic field-medical format. Not a real document and not derived from any real record.

[CONFERENCE REFERENCE] The meeting named above is a real, publicly announced scientific congress and is used here only as the setting of this fictional record. No session content, abstract, poster, or presented data from that meeting is reproduced or paraphrased; every person and conversation below is invented.

Congress Attendance Report — American Academy of Neurology Annual Meeting (AAN) 2021

Meeting period: April 2021 | Author: Priya Menon

Scope: unsolicited conversations held on-site. Session content and presented data are deliberately not reproduced here.

Conversation — Daniel Whitcomb, MD (Northbrook Neurology Institute)

Introduced by a colleague from the same region and talked for a few minutes. Asked whether I had seen a session summary from a colleague; I had not, and offered nothing in return. Referral patterns into this clinic have changed, and the refractory group is a bigger share than it was. Asked whether I would be at the meeting later this year.

Conversation — Malika Danforth, MD (Riverstone Comprehensive Epilepsy Clinic)

Introduced by a colleague from the same region and talked for a few minutes. The composition of the pediatric-to-adult transfers has shifted over the last year or two. Monitoring intervals during titration are set by clinic routine here rather than by protocol. Mentioned a review that circulated on the service but did not raise any specific finding.

Conversation — Jaromir Merriwether, MD (Sable Creek Medical Group)

A hallway conversation during the afternoon break. Described how the refractory group is triaged now compared with before. Benefit verification is the step that most often delays a start here. No product discussion this time. Asked about safety and confirmed nothing to report.

Conversation — Ilse Beauchamp, MD (Harborlight Epilepsy Center)

A short exchange over the coffee queue. Titration habits came up in general terms; no specifics were discussed and none were offered. The new charting templates is still being sorted out, and that took up the first part of the conversation. Mentioned a guideline update discussed at journal club but did not raise any specific finding.

Conversation — Hadley Wycliffe, MD (Northbrook Neurology Institute)

Approached me rather than the other way around, which is worth noting. The composition of newly referred patients has shifted over the last year or two. Talked through long-standing patients in general terms, without any specific case. No product discussion this time. Asked about safety and confirmed nothing to report.

Conversation — Felicity Fairweather, MD (Northgate Neurology Partners)

The conversation happened while waiting for a session room to clear. Talked through long-standing patients in general terms, without any specific case. Most of the discussion was about the phone triage line, which is still being sorted out.

Conversation — Viraj Adeyemi-Cole, MD (Thornfield Regional Epilepsy Program)

Stopped to talk on the way out of a session block, with no agenda on either side. Closed there — nothing actionable this visit. Referral patterns into this clinic have changed, and the pediatric-to-adult transfers is a bigger share than it was.

Conversation — Alice Nguyen-Barr, MD (Old Dominion Neurology Group)

A hallway conversation during the afternoon break. Referral patterns into this clinic have changed, and the pediatric-to-adult transfers is a bigger share than it was. This practice is still building familiarity with the product and had no clinical points to raise.

Conversation — Renata Cavanaugh, MD (Northbrook Neurology Institute)

We spoke standing near the exhibit aisles, briefly. Formulary placement was the only product-adjacent topic, and only in general terms. Benefit checks takes longer than the clinical decision, which came up as a general frustration. Asked about my travel schedule, since visits keep landing on the busiest clinic days.

Conversation — Quentin Lammermoor, MD (Riverstone Comprehensive Epilepsy Clinic)

A hallway conversation during the afternoon break. No materials requested. Safety items asked about and confirmed none. Talked through newly referred patients in general terms, without any specific case. Described how the older half of the panel is triaged now compared with before.

Follow-ups have been logged in the interaction system.